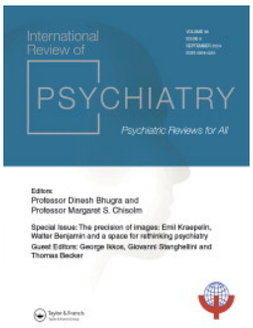

International Review of Psychiatry

ISSN: 0954-0261 (Print) 1369-1627 (Online) Journal homepage: www.tandfonline.com/journals/iirp20

The precision of images: Emil Kraepelin, Walter Benjamin and a space for rethinking psychiatry

George Ikkos, Giovanni Stanghellini & Thomas Becker

To cite this article: George Ikkos, Giovanni Stanghellini & Thomas Becker (2024) The precision of images: Emil Kraepelin, Walter Benjamin and a space for rethinking psychiatry, International Review of Psychiatry, 36:6, 553-556, DOI: 10.1080/09540261.2024.2382022

To link to this article: https://doi.org/10.1080/09540261.2024.2382022

Published online: 29 Jul 2024.

Submit your article to this journal

Article views: 1155

View related articles

View Crossmark data

Full Terms & Conditions of access and use can be found at
https://www.tandfonline.com/action/journalInformation?journalCode=iirp20

INTERNATIONAL REVIEW OF PSYCHIATRY
2024, VOL. 36, NO. 6, 553–556
https://doi.org/10.1080/09540261.2024.2382022

Taylor & Francis
Taylor & Francis Group

Check for updates

EDITORIAL

The precision of images: Emil Kraepelin, Walter Benjamin and a space for rethinking psychiatry

The year 2026 marks the centenary of the death of Emil Kraepelin (1856–1926) and with this also, we suggest, the end of psychiatry's long 20th century 1899–1926. This is so both because of challenges to his legacy from within psychiatry and because of epochal changes in society since his times. Here we will very briefly summarise these, justify the relevance of Walter Benjamin (1892–1940) and introduce the collection of papers in this issue.

Emil Kraepelin's legacy

Arguably the most significant figure in 19th and 20th century psychiatric research and the history of psychiatry, Kraepelin's delineation, in the 6th edition of his *Handbook of Psychiatry* in 1899, of the syndrome of dementia praecox which later came to be called schizophrenia (Kendler, 2020), his argument for its separation from manic-depressive insanity and, perhaps even more, his very idea that the basis of psychiatry as a science should be a rigorous classification of psychopathological syndromes set the foundations of psychiatry for the whole of the 20th century.

Though 'not doomed', both the foundations and evolution of psychiatry have been consistently challenged and remain so (Ikkos, 2023). Importantly, Kraepelin's legacy has been questioned from within psychiatry as well as beyond, both early on and more recently. His was a world in which the classification of mental disorders was based on careful clinical distinctions, i.e. on the observation of symptoms and psychopathological phenomena. However, now this art has fallen into disuse in the teaching of psychiatry (Fiorillo et al., 2016). With respect to his flagship concept of schizophrenia, a recent challenge has come from new discoveries in genetics, a discipline dear to his thinking (Marsman et al., 2020; Nasrallah et al. 2024). Indeed, the very distinction between schizophrenia and what is now called bipolar disorder has become increasingly controversial due to findings in this discipline (Craddock & Owen, 2010). As a result, though many doubt the benefits of the deconstruction of the Kraepelinian dichotomy (Stanghellini & Raballo, 2015), others have gone as far as to advocate against use of the label of 'schizophrenia' at all, preferring that of 'psychosis'. These latter colleagues argue that this will help us to better focus on the day-to-day challenges shared by people suffering from different 'severe' mental disorders (Parabiaghi et al. 2006).

Psychiatry in social and historical contexts

Antecedents of psychiatry go as far back as shamanism, and inpatient mental health facilities could be found in the Arab world as far back as the 9th century in the common era. Early forebears of institutions for people with mental health conditions also appeared in medieval Europe because of the Arab legacy and in Christian charity (Ikkos, 2010). However, it is not until the era of European Absolutism, especially during the 18th century, that one finds the increasing spread of madhouses and other institutions which provided the material foundations for the emergence of psychiatry as a profession and as a social apparatus aimed at treatment, but also surveillance and control. Indeed, the emergence of psychiatry itself as a profession is closely tied in with the European transition from the absolutist state to the European capitalist political economy, imperial states and bourgeois democracies (Iliopoulos, 2019). Within the context of Enlightenment Reason and scientific and technological progress, society sought to found the very idea of 'man' as one based on rationality, production, self-control, compliance, etc. Caught in this context, there has been a range of motivations (religious, welfarist, coercive, etc.), epistemologies (Darwinian, social, psychoanalytic, etc.) and practices (state and private, institutional and community, etc.). Along the way there have been severe tragedies (Degeneration theory, Eugenics, diagnostic excess and others) which must never be discounted.

To headline two centuries of relevant developments: if the 19th century of psychiatry was inextricably linked to the asylum and degeneration theory, and the early part of the 20th with the two world wars, the Nazi euthanasia programme and the rise of psychoanalysis and the jargon of interiority. In economically advanced countries since the second world war we have seen the beginnings

This article has been corrected with minor changes. These changes do not impact the academic content of the article.

of deinstitutionalisation and community psychiatry and since the 1980s the hegemony of biological psychiatry. Policies of deinstitutionalisation and community care came about in the context of post war social and Christian democracy, social liberalisation and the clamour for civil rights. Crucial as well, however, have been financial factors (Scull, 1977). Social factors have been hugely determinant throughout the history of psychiatry and financial factors have been detrimental to deinstitutionalization and community care (Ikkos & Bouras, 2024). This has been very unfortunate, especially as at the same time as deinstitutionalisation, biological psychiatry since the 'decade of the brain' (1990s) has failed to match advances compared to other medical specialties.

Though there is continued policy emphasis on community psychiatry, the principles and practice of which we laud, and all three of us have advocated for and participated in, the subsequent development of neoliberal economy and evolution of technology, including digital technology (Stanghellini & Leoni, 2020), have disrupted its course. Necessary though community psychiatry has been, Ikkos and Bouras (2024) have questioned its outcomes, to say the least, and have argued that we have entered a new era, that of 'Metacommunity psychiatry and mental healthcare'. This sets new tasks for the profession in terms of theory, science, practice and, last but not least, *activism*.

Others have commented on fundamental transformations in society more broadly. Reckwitz (2017) has argued that we have moved on from modern societies to societies of singularities. This is not the place to detail his well evidenced argument, but we note three of the points he makes. One is that of extreme individualism. The second is what he labels as the profound culturalization of society. And the third is the abundance of cultural products, in contrast to key resource scarcity characterizing political economies of the past. This cultural abundance is associated with unpredictability, instability and the success of 'singularities' at the expense of most of their fellow citizens. It is Reckwitz' idea of culturalization and the dialectic of singularity-failure that make Walter Benjamin, whose philosophy and methodology are very different from a mainstream utilitarian and capabilities-based thinking, relevant as a resource for psychiatry.

One of the ways psychiatrists have attempted to meet some of the discipline's challenges, dilemmas and conundrums has been to adopt the rhetoric of the biopsychosocial model. This has been laudable in attempting to curb the excesses of so-called biological or social or psychoanalytic or other restrictive versions of psychiatry. However, it has never been free of criticism for a certain vagueness at the heart of it (Bolton, 2022). Fair observers readily concede that in daily use it is heavily biased towards the bio, perhaps also the psych but not the social (Williamson, 2022). GI and GS have recently argued that it is particularly in clinical practice, with the patient in front of the clinician, that the biopsychosocial model fails woefully in appreciating and bringing into play the full force of the social (Ikkos & Stanghellini, 2024).

It is essential that psychiatrists be aware of the 'social' not only as a causal element in the pathogenesis of psychopathological conditions. We need to be more generally aware of the constraints surreptitiously imposed on therapeutic practices by the social, cultural and political context, of the ways in which psychopathological pictures are transformed and changed within this context, of the ways context influences the diagnostic categories in use, and in general of the ways it shapes the very ideas of man, health and illness to which psychiatrists themselves most often conform. It is the epistemological and, ultimately, ethical shortfall of awareness shared by both biological and social psychiatry that Benjamin offers us tools to address.

So, why Walter Benjamin?

Benjamin taught us a new, critical way of thinking about history. Not as the linear, continuous and progressive evolution towards the future, but as an opportunity for critical awakening and for radical change.

The *pars destruens* of the Benjaminian concept of 'history' includes his critique of the excessive trust in the myth of 'progress' understood as the constant, unstoppable, infinitely perfectible advancement of human affairs, especially with respect to knowledge and science. This faith in the march towards a better destiny gives rise to the comforting desire of many to swim uncritically with the current. Benjamin's critique also stigmatizes the naive belief that adhering to the course of history thus conceived is not itself a political gesture – even if qualified as 'progressive' and not 'conformist'. This conformity to the 'magnificent and progressive destiny' of psychiatry's progress suffocates the 'revolutionary' vocation of psychiatrists, especially young psychiatrists, driven at the beginning of their careers by ideals of justice, equality, recognition, respect for diversity, if not outright by the idea (perhaps a little romantic) of the disruptive and potentially 'revolutionary' force of deviance and mental illness.

The *pars adstruens* of Benjamin's teaching shows the need for psychiatrists to know history and critically question the way in which it is usually narrated, i.e. its rhetoric. Is the temporality of history progressive, linear, homogeneous, continuous? Or does history proceed in fits and starts? And does its process not feed on fortuitous occasions? And this concerns not only the macro-history of the discipline called 'Psychiatry' – but the history of the patients themselves, which is often reduced to an 'anamnesis' that seeks the antecedents and causes of the present in the past. History, Benjamin teaches, is made up of episodes 'saturated with tension', much more than continuous and linear developments.

In contrast, what is the best, the most effective way to 'tell' this story? As a rule, psychiatrists are educated to use concepts, 'operationalized' linguistic formulas, in the name of the requirements considered indispensable for the progress of medical science. Benjamin shows the usefulness of images, alongside concepts. He also shows how illusory it is to try to resolve the ambiguities of history with dichotomous distinctions, since history is intrinsically made of ambiguity, or as he says of 'dialectical tensions', that is of 'eddies', 'refluxes', 'stagnations' and 'calms'. Psychiatrists being aware of this perspective means realizing that, like our patients, we are immersed in this flow and that it is our responsibility to understand it, decide in which direction to go and seize the right opportunity to do so.

Emil Kraepelin, Walter Benjamin and a space for rethinking psychiatry

This special issue attempts to address some aspects of the work of two seminal but distant scientists and intellectuals of the 20th century, Emil Kraepelin and Walter Benjamin. Clearly, the former is of paramount importance to the field of psychiatry and mental health sciences in the 20th century (and beyond). And is there any indication that the latter (Benjamin) might be relevant too? What are the building blocks of this special issue of *International Review of Psychiatry*?

Through an exploration of the work and ideas of Emil Kraepelin, Karl Jaspers and Arthur Kronfeld, Paul Hoff reminds us that such a fundamental and long discussed concept as mental illness remains ill-defined and that we may benefit from past as well as modern discourse around it. Becker and Hoff then review and contrast the intellectual heritage, personal commitments and methodologies of Emil Kraepelin and Walter Benjamin. George Ikkos, Giovanni Stanghellini and Alastair Morgan, in a series of three linked papers, provide a detailed introduction into Benjamin, with respect to his world, history and ideas and their relevance to the history of psychiatry, professional identity of psychiatrists and clinical practice. Specifically, Ikkos, Stanghellini and Morgan write about Benjamin's personal and philosophical formation, political commitments and such key ideas of now-time and dialectical images. Stanghellini utilises Benjamin's image of the ragpicker, to emphasise the importance of psychiatrists attending to phenomena that others forget or discard and to think dialectically. Morgan also employs Benjamin's concept of the dialectical image, this time to illuminate a new way of seeing and approaching psychosis.

The next two papers form a pair in referring significantly not only to Benjamin but his contemporary Italian political theorist and communist activist Antonio Gramsci. Becker et al. offer an introductory comparison of these two highly significant cultural political 20th century figures and discuss their relevance to thinking and acting for mental health reform, including active involvement of service users. The theories of Gramsci and Benjamin came together in the work of radical Italian reforming psychiatrist Franco Basaglia. This pertains more so to Gramsci than Benjamin, but Brencio makes innovative use of Benjamin's ideas to help understand Basaglia's Gramsci-inspired practical activities. The final three papers return to images, a key concept in our series. Esposito and Stanghellini look at the image of the self in hysteria, Gallese focuses on images of the self in the digital age, and Correale looks at the clinical concepts of structuring images and epiphany. Finally, reflecting from the heart of Walter Benjamin's concerns, Stanghellini (2024b examines critically the current vogue for narrative in medicine and psychiatry, alerts us to the value the "fragment" and offers a way to think about it in the context of the clinical consultation.

Conclusion

In dialogue with the wide range of other sciences and professions with a stake in mental health, psychiatrists may discover a new scientific perspective and methodology in Benjamin's work. Applying some of his thinking and of those that have followed him to mental health science (and practice) may provide vistas to better understand the multitude of experiences that mental suffering encompasses. His non-linear way of 'thinking mental ill health' may be seen as a promising pathway towards updating and broadening the European enlightenment traditions with reference to psychiatry and in this way enriching the thriving field of mental health science.

Acknowledgements

Thomas Becker, European Editor of *International Review of Psychiatry*, proposed the theme for this special issue and has acted as its lead editor. He has contributed the first draft of the present editorial. George Ikkos and Giovanni Stanghellini contributed to major revision and expansion. We are grateful to the Editor in Chief and the Editorial Board for their ready acceptance of our proposal for 'The Precision of images: Emil Kraepelin, Walter Benjamin and a space for rethinking psychiatry' and to all the authors for their excellent contributions to this special issue.

References

Bolton, D. (2022). Looking forward to a decade of the biopsychosocial model. *BJPsych Bulletin*, *46*(4), 228–232. https://doi.org/10.1192/bjb.2022.34

Craddock, N., & Owen, M. J. (2010). The Kraepelinian dichotomy – going, going … but still not gone. *British Journal of Psychiatry*, *196*(2), 92–95. https://doi.org/10.1192/bjp.bp.109.073429

Fiorillo A., Sampogna, G., Del Vecchio, V., Luciano, M., Ambrosini, A., & Stanghellini, G. (2016). Education in psychopathology in Europe: Results from a survey in 32 countries. *Acad Psychiatry*, *40*, 242–248. https://doi.org/10.1007/s40596-015-0333

Ikkos, G. (2010). Psychiatry, professionalism and society; a note on past and present. In D. Bhugra, A. Malik, & G. Ikkos (Eds.), *Psychiatry's Contract with Society*. Oxford: Oxford University Press.

Ikkos G. (2023). Not doomed: Sociology and psychiatry, and ignorance and expertise. *BJPsych Bulletin*, *47*(2), 90–94. https://doi.org/10.1192/bjb.2022.60

Ikkos, G., & Bouras, N. (2024). Metacommunity: The current status of psychiatry and mental healthcare and implications for the future. *BJPsych International*. Online, 1–4. https://doi.org/10.1192/bji.2024.15

Ikkos, G., & Stanghellini, G. (2024). *Psychiatric Practice and the Living Force of the Social in the Biopsychosocial*, in press.

Iliopoulos, J. (2019). *The History of Reason in the Age of Madness: Foucault's Enlightenment and a Radical Critique of Psychiatry*. London: Bloomsbury Academic.

Kendler, K. S. (2020). The development of Kraepelin's concept of dementia praecox: A close reading of relevant texts. *JAMA Psychiatry*, *77*(11), 1181–1187. https://doi.org/10.1001/jamapsychiatry.2020.1266

Marsman, A., Pries, L.-K., ten Have, M., de Graaf, R., van Dorsselaer, S., Bak, M., Kenis, G., Lin, B. D., Luykx, J. J., Rutten, B. P. F., Guloksuz, S., van Os, J. (2020). Do current measures of polygenic risk for mental disorders contribute to population variance in mental health? *Schizophrenia Bulletin*, *46*(6), 1353–1362. https://doi.org/10.1093/schbul/sbaa086

Nasrallah, H., Akbarian, S., Carpenter, W. T.Jr., DeLisi, L. E., Gaebel, W., Green, M. F., Gur, R. E., Heckers, S., Kane, J. M., Malaspina, D., Meyer-Lindenberg, A., Murray, R., Owen, M., Smoller, J. W., Yassin, W., & Keshavan, M. (2024). The schizophrenia syndrome, circa 2024: What we know and how that informs its nature. *Schizophr Res*, *264*, 1–28. https://doi.org/10.1016/j.schres.2023.11.015

Parabiaghi, A., Bonetto, C., Ruggeri, M., Lasalvia, A., & Leese, M. (2006). Severe and persistent mental illness: A useful definition for prioritizing community-based mental health service interventions. *Soc Psychiatry Psychiatr Epidemiol*, *41*(6), 457–463. https://doi.org/10.1007/s00127-006-0048-0

Reckwitz, A. (2017). *The Society of Singularities*. Polity.

Scull, A. (1977). *Decarceration, Community Treatment and the Deviant; a Radical View*. Prentice Hall.

Stanghellini, G., & Leoni, F. (2020). Digital phenotyping: Ethical issues, opportunities, and threats. *Front. Psychiatry*, *11*, 473. https://doi.org/10.3389/fpsyt.2020.00473

Stanghellini, G., & Raballo, A. (2015). Differential typology of delusions in major depression and schizophrenia. A critique to the unitary concept of 'psychosis'. *Journal of Affective Disorders*, *171*, 171–178. https://doi.org/10.1016/j.jad.2014.09.027

Williamson, S. (2022). The biopsychosocial model: not dead, but in need of revival. *BJPsych Bulletin*, *46*(4), 232–234. https://doi.org/10.1192/bjb.2022.29

George Ikkos
Department of Liaison Psychiatry, Royal National Orthopaedic Hospital NHS Trust, Stanmore, UK

Giovanni Stanghellini
Department of Health Sciences, University of Florence, Florence, Italy
Centro de Estudios de Fenomenologia y Psiquiatria, Universidad 'Diego Portales', Santiago, Chile

Thomas Becker
Department of Psychiatry, University of Leipzig Medical Center, Leipzig, Germany
ikkos@doctors.org.uk

© 2024 Institute of Psychiatry and Johns Hopkins University